

## Pediatric P-06 Pediatric Shock

**Priorities**

Primary Assessment  
ABCs  
Early notification of receiving hospital  
Identify causes of shock  
Treat cardiac dysrhythmias per appropriate protocol

Stabilize airway using BLS adjuncts if necessary

**Oxygen** – titrate to SpO<sub>2</sub> >95%

Cardiac Monitor

IV/IO Access

Blood Glucose Check

**NS 20mL/kg bolus IV/IO**

May repeat once for persistent age-adjusted hypotension

Contact base for further orders for additional NS bolus

**For blood glucose <50mg/dl in patients <1 year old**

**Dextrose 10% 0.5g/kg (5mL/kg) IV/IO**  
to max dose of 25g (250mL)

**For blood glucose <60m/dl in patients 1-14 years old**

**Dextrose 10% 0.5g/kg (5mL/kg) IV/IO**  
to max dose of 25g (250mL)

**If no IV/IO can be established**

**Glucagon 0.5mg IM** for patients <6 years old  
or

**Glucagon 1mg IM** for patients ≥6 years old  
or

1 tube **Oral Glucose** if patient has gag reflex  
and is able to swallow

**Dextrose Dosage Calculations**

Dextrose 10% is the preferred concentration. If Dextrose 10% is not available, please use the doses below for other concentrations.

D10% 5mL/kg (max 250mL)  
0.5g/kg (max 25g) = D25% 2mL/kg (max 100mL)  
D50% 1mL/kg (max 50mL)

**DISRUPTED COMMUNICATIONS**

In the event of a “disrupted communications” situation, Solano County Paramedics may utilize all portions of this treatment protocol without Base Hospital Contact as needed to stabilize an immediate patient.